

Moderate (Procedural) Sedation

Policy Number	POL-PH-037	Owner / Department	Anesthesia / Medical Staff
Effective Date	2024-02-01	Version	2.0
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Approved By	Pharmacy & Therapeutics Committee	Applies To	All credentialed providers and nurses administering or monitoring moderate sedation outside the operating room

1. Purpose

To ensure safe patient selection, monitoring, and rescue during moderate (procedural) sedation, consistent with the American Society of Anesthesiologists (ASA) continuum of depth of sedation and The Joint Commission moderate-sedation standards.

2. Scope

Applies to moderate sedation administered outside the operating room by credentialed non-anesthesiologist providers. It does not govern minimal sedation (anxiolysis) or deep sedation/general anesthesia, which require anesthesia privileging.

3. Definitions

- **Moderate sedation (“conscious sedation”)** — drug-induced depression of consciousness in which the patient responds purposefully to verbal commands, alone or with light tactile stimulation.
- **Rescue** — the ability to recognize and manage a level of sedation deeper than intended; providers must be able to rescue a patient from the next deeper level.
- **Capnography** — continuous monitoring of exhaled carbon dioxide to detect hypoventilation and apnea.
- **ASA Physical Status** — the ASA I–VI classification used to assess pre-procedure risk.

4. Policy and Procedure

A pre-sedation assessment (history, airway evaluation, ASA physical status, and NPO status) is completed and documented before sedation. A qualified individual whose sole responsibility is monitoring the patient must be present throughout; the provider performing the procedure does not also serve as the monitor.

1. Complete and document the pre-sedation assessment, airway exam, ASA status, and NPO verification.
2. Confirm emergency equipment, suction, oxygen, reversal agents, and resuscitation support are immediately available.
3. Monitor oxygenation (pulse oximetry), ventilation (capnography), level of consciousness, blood pressure, and heart rate continuously.
4. Titrate sedatives/analgesics to the lightest effective depth, reassessing after each increment.
5. Recover the patient to defined discharge criteria before release.

IMPORTANT — If a patient progresses to a deeper level of sedation than intended (unresponsive to verbal/tactile stimulation, airway compromise, or apnea), stop sedation, support the airway and ventilation immediately, call for anesthesia/code assistance, and administer reversal agents per protocol. The team must be able to rescue from deep sedation.

5. Sedation Continuum and Reversal Agents

Level	Responsiveness	Airway
Minimal (anxiolysis)	Normal response to verbal stimuli.	Unaffected.
Moderate (target)	Purposeful to verbal / light tactile.	No intervention; adequate ventilation.
Deep	Purposeful after repeated or painful stimulation.	May need intervention.
General anesthesia	Not arousable, even with pain.	Often requires intervention.

Reversal agents must be immediately available: **naloxone** for opioid-induced respiratory depression and **flumazenil** for benzodiazepine over-sedation.

6. Severity & Escalation

Classify sedation events on the POL-RM-013 scale; when a medication error contributes, grade it by the NCC MERP index per POL-PS-003.

Severity	Example	Notify / timeframe
SEV-1	Respiratory arrest, unplanned intubation, or death during sedation.	Provider, Nursing Supervisor & Risk Mgmt within 1 hour.
SEV-2	Oversedation requiring reversal, bag-mask ventilation, or prolonged recovery.	Provider & Risk Mgmt within 4 hours.
SEV-3 / Near Miss	Transient desaturation self-corrected; monitoring lapse caught with no harm.	Event report within 24 hours.

7. Documentation Requirements

- Pre-sedation assessment, airway exam, ASA physical status, and NPO verification.
- Medications, doses, and times, and the monitoring individual's name.
- Continuous vitals, oxygenation, and capnography at defined intervals.
- Any reversal agent given and the patient's response.
- Recovery scoring and that discharge criteria were met.

8. References

- ASA Continuum of Depth of Sedation: Definition of General Anesthesia and Levels of Sedation/Analgesia.
- ASA Practice Guidelines for Moderate Procedural Sedation and Analgesia.
- The Joint Commission moderate-sedation standards (Provision of Care).

- CMS Conditions of Participation, 42 CFR 482.52 (anesthesia services).
- Related: POL-RM-013 (severity & reporting); POL-PS-003 (medication-error reporting).